

# PATIENT HANDOFF NOTE

## ■■ CRITICAL PATIENT INFORMATION ■■

|               |                        |
|---------------|------------------------|
| Patient Name: | Marcus Client          |
| Patient ID:   | 14                     |
| Handoff Date: | October 16, 2025 11:17 |
| Shift From:   | Day Shift              |
| Shift To:     | Night Shift            |
| Created By:   | Staff ID: 11           |

## PATIENT OVERVIEW

Mr. Client is an adult patient with an unknown age and gender. He presents with a primary current concern of gastritis, diagnosed on 2025-10-16, reporting stomach pain, burning sensation post-meals, bloating, and mild nausea for one week. He also has a history of sadness/depression, as noted in recent appointments. Past medical history includes Cholecystectomy (2012) and Childhood Asthma.

## CURRENT CONDITION

Mr. Client is currently stable. Latest vital signs are within normal limits: BP 122/78, HR 68, Temp 98.2°F, O2 99% on room air. He reports persistent stomach pain as described above but denies vomiting, blood in stool, or weight loss. No acute respiratory distress related to asthma. Mental status is not fully assessed but "sadness, depression" were noted recently.

## ACTIVE PROBLEMS

\* \*\*Gastritis:\*\* Acute presentation with epigastric burning pain, bloating, and nausea. \* \*\*Depression/Sadness:\*\* History noted in recent appointments, requiring further evaluation. \* \*\*History of Childhood Asthma:\*\* No active exacerbation noted. \* \*\*History of Cholecystectomy (2012):\*\* Relevant for abdominal pain differential, but ruled out by gastritis assessment.

## RECENT CHANGES (24-48 HRS)

On 2025-10-16, Mr. Client was assessed with Gastritis due to a one-week history of burning stomach pain after meals, bloating, and mild nausea. This is the most recent significant clinical development.

## PENDING TASKS

\* Further workup for gastritis (e.g., H. pylori testing, PPI trial) is likely indicated. \* Comprehensive mental health assessment and follow-up for reported sadness/depression. \* No specific outstanding orders, tests, or procedures were provided in the patient's information.

## ■■ IMPORTANT ALERTS

\* \*\*Allergies:\*\* Penicillin, Aspirin, Seasonal Pollen. \* \*\*Precautions:\*\* None specified. Consider suicide risk assessment given history of depression.

## CARE PLAN

\* \*\*Gastritis Management:\*\* Continue to monitor symptoms (pain, nausea, bloating). Provide education on dietary modifications (avoiding spicy, fatty, acidic foods, caffeine, alcohol). Consider initiating a proton pump inhibitor (PPI) or H2 blocker as per protocol/provider orders. If symptoms persist or worsen, consider further diagnostics such as endoscopy. \* \*\*Mental Health:\*\* Initiate a full mental health assessment to evaluate the extent of depression/sadness. Consult with a mental health specialist or social worker for ongoing support and potential treatment (e.g., therapy, pharmacotherapy). \* \*\*General:\*\* Maintain stable vital signs. Monitor for any changes in condition or new symptoms.

## FAMILY COMMUNICATION

No information regarding family contact, involvement, or communication preferences was provided.

## ADDITIONAL NOTES

Patient requested pain medication at 2pm

## SPECIAL INSTRUCTIONS

Monitor vitals every 2 hours

## HANDOFF ACKNOWLEDGMENT

Receiving Staff Signature:

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Date/Time:

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Printed Name:

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